Diabetes Examination — OSCE Checklist

Deakin MD2 · ELC3 · Time: 10–12 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Vitals → Hands → Face → Eyes → Neck/Chest → Abdomen → Lower limbs**. *Look for* **microvascular** *(eyes, kidneys, nerves) and* **macrovascular** *(heart, carotids, legs) complications.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain and gain consent; ask about pain
☐	Position and expose as for CV exam, plus abdomen and lower limbs (keep the patient covered and comfortable)

2. Examination

	Step	What to do / say	Significance
General inspection			
☐	Age, body habitus	Obesity, central adiposity; cachexia	T2DM, metabolic syndrome
☐	Acute distress	Hyperventilating, unwell (Kussmaul breathing); confused, anxious, sweaty	DKA; hypoglycaemia
☐	Gait	Observe as the patient enters	Neuropathy, PVD, foot problems
Vital signs and measurements			
☐	BP lying and standing	Postural drop	Autonomic neuropathy, dehydration
☐	HR, RR, SpO_2, temp	Full set	Sepsis, DKA
☐	BGL ± ketones	Ketones if BGL elevated	Ketones >0.6 mmol/L = red flag
☐	Weight, height, BMI, waist	If appropriate and able	Cardiometabolic risk
Hands and arms			
☐	Fingertips	Finger-prick marks; sites rotated?	Self-monitoring
☐	Dexterity	OA, limited joint mobility	Ability to self-inject / test
☐	Upper arm	Continuous glucose monitor (CGM)?	
Face			
☐	Endocrine facies	"Moon face"	Cushing's (secondary diabetes)
☐	Mouth	Infection: bacterial or Candida (white plaques)	Poor glycaemic control
Eyes			
☐	Visual acuity	Snellen chart, glasses on; each eye	Retinopathy, maculopathy, cataract
☐	Eye movements*	If clinically indicated	CN III / VI mononeuropathy
☐	Fundoscopy	Red reflex, disc, vessels, macula (note undilated-pupil limits)	Retinopathy, cataract
Neck and chest			
☐	Carotids	Auscultate for bruits	Carotid atherosclerosis
☐	Acanthosis nigricans	Brown velvety skin: back of neck, axillae, folds	Insulin resistance
☐	Precordium	Auscultate heart; ± lung bases if cardiac concern	Heart failure, IHD
Abdomen			
☐	Injection sites	Clean? Rotated? Lipohypertrophy or lipoatrophy	Erratic insulin absorption
Lower limbs — inspect (incl. backs of legs, between toes, soles)			
☐	Gait and footwear	Gait; shoe type and fit	Pressure areas
☐	Nails and foot care	Nail condition; impingement on adjacent toes	
☐	Interdigital	Maceration, tinea pedis	Infection portal
☐	Skin	Dry fissured skin, atrophy, callus, necrobiosis lipoidica	Neuropathy, high pressure
☐	Pressure areas, ulcers	Heels, metatarsal heads, toes: ulcer, crack, erythema; depth?	Ulcer to tendon/bone = red flag
☐	Trophic changes	Hair loss, shiny skin	PVD
☐	Bone architecture	High arch, clawed toes, bunions, muscle wasting, Charcot joint	Motor / sensory neuropathy
Lower limbs — vascular			
☐	Pulses	Distal → proximal: DP, PT, popliteal, femoral	PVD
☐	Perfusion	Capillary refill in toes; temperature of limbs	Critical limb ischaemia = red flag
Lower limbs — neurological			
☐	Monofilament (10 g)	Eyes closed; several plantar sites each foot (hallux, 1st/3rd/5th met heads †)	Loss of protective sensation
☐	Vibration	128 Hz fork at base of great toe; move proximally if absent	Dorsal column / large fibre — most sensitive
☐	Proprioception	Great toe, held by its sides; move proximally if absent	Sensory ataxia
☐	Reflexes	Plantar → ankle → knee	Reduced ankle jerks in neuropathy

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion: "I would perform urinalysis (ketones, leucocytes, nitrites ± albumin), and check HbA1c, UEC/eGFR, urine ACR and lipids †."
☐	Present by complication: glycaemic control clues → microvascular (eyes, nerves, kidneys) → macrovascular (carotids, heart, peripheral pulses) → feet

4. Quick reference

Complication	Examination findings
Retinopathy	Microaneurysms, dot-blot haemorrhages, hard exudates → cotton-wool spots → new vessels †
Neuropathy	Stocking sensory loss, ↓ vibration, absent ankle jerks, Charcot foot
Nephropathy	Oedema, ↑ BP; needs urine ACR †
Macrovascular	Carotid bruit, absent pulses, trophic changes, ↓ cap refill

#	Red flags (Deakin guideline)
1	Symptomatic hyperglycaemia ± dehydration
2	Ketosis >0.6 mmol/L or ketoacidosis
3	Severe infection / sepsis
4	Ulcer with depth to tendon or bone
5	Critical limb ischaemia
6	Pregnancy